



Most of the impetigo we see is non-bullous and caused by staph Aureus, some strep. pyogenes or both.

Management

Rule out rare but not-to-be-missed red flags:

Red flags

Systemically unwell

High risk of complications
(e.g. immunocompromised)

Bullous impetigo

Larger vesicles/pustules, often >1cm, that rupture leaving thin, flat, yellow-brown crust

Give oral
antibiotics
(doses overleaf)
and
consider
admission

But the majority will have...

Non-bullous impetigo

Thin-walled vesicles/pustules that rupture quickly forming golden-brown crust

Localised non-bullous impetigo

Widespread non-bullous impetigo

**Consider hydrogen peroxide 1% cream
(Crystacide 1%) 2–3x a day for 5d.**

*If unsuitable (e.g. widespread, around the eyes),
offer topical antibiotic.*

**If no improvement with topical hydrogen
peroxide, offer antibiotics:**

- Topical if disease is localised.
- Oral if disease has become more widespread.

Offer topical or oral antibiotics.

Base decision on:

- Patient preference.
- Size of infected area: oral therapy more practical for larger areas.
- Recent topical antibiotic use: resistance can develop rapidly if repeated use of topical antibiotics or extended courses.

If topical antibiotics used but there is no improvement:

- Consider sending a swab.
- Offer oral antibiotics.

Hygiene measures

Good hygiene reduces spread to other parts of the body and to other people:

- Wash affected areas with soap and water.
- Wash hands regularly, in particular after touching a patch of impetigo.
- Avoid scratching affected areas.
- Avoid sharing towels, face cloths and other personal care products, and thoroughly clean potentially contaminated toys and play equipment.

School exclusion

- Exclude until lesions are crusted and healed, or 48h after starting antibiotics (*see note in article on some variation in guidance in this area*).

Swabs

Swab if:

- Impetigo worsening after oral antibiotics.
- Recurrent impetigo (send skin swab and consider sending nasal swab).

Consider swabbing if:

- No improvement/worsening after topical antibiotics (swab and give oral antibiotics).

2. Swabs, doses, referral, troubleshooting

Treatment doses and duration

- **Treat for 5d in most**, but a 7d course may be given in more severe/extensive disease.
- Do NOT give topicals and orals together (increased risk of resistance).
- If MRSA suspected (rare): consult local microbiologist.

Topical therapies

Topical antiseptic	Crystacide	2–3x a day for 5d. Will bleach clothes/towels etc.!
First-line topical antibiotic	Fusidic acid	Apply 3x a day for 5d.
Alternative topical antibiotic	Mupirocin 2%	Apply 3x a day for 5d (slightly greater risk of rash with this compared with fusidic acid).

First-line oral

Flucloxacillin	1 month–1 year	2–9y	10–17y	Adults
	62.5–125mg 4x a day	125–250mg 4x a day	250–500mg 4x a day	500mg 4x a day

If unable to take flucloxacillin

Clarithromycin (not in pregnancy)	<8kg	8–11kg	12–19kg	20–29kg	30–40kg	12y–adults
	7.5mg/kg twice daily	62.5mg twice daily	125mg twice daily	187.5mg twice daily	250mg twice daily	250mg twice daily (500mg in severe disease)
Erythromycin (in pregnancy)	250–500mg 4x a day					

NICE's rationale for treatment choices

- **No treatment (topical disinfectants, topical antibiotics, oral antibiotics) has been shown to be more effective than another.** Indeed, topical steroids and topical antifungals have been shown to be equally effective as antibiotics and disinfectants, but NICE did not opt for these treatments (presumably because they do not fit with the known pathogenesis).
- **Why topical antiseptics first line?** NICE opted for a treatment that is MORE expensive (an extra £3.50/patient/treatment) for reasons of antibiotic stewardship.
- **Why this disinfectant, not others?** Other (cheaper) topical antiseptics are available but are not recommended because of lack of evidence. The evidence of effectiveness of topical disinfectants comes from a single trial of just 256 children (fusidic acid vs. hydrogen peroxide 1%).
- **Topical vs. oral antibiotics?** There was no significant difference in effectiveness between oral and topicals.

Referral

Refer if:

- Features suggesting more serious conditions, e.g. cellulitis.
- Widespread impetigo in immunocompromised person.

Consider referral if:

- Recurrent impetigo.
- Bullous impetigo (especially those <1y).
- Systemically unwell.
- High risk of complications.

Troubleshooting

Deterioration after/ despite treatment

Consider other diagnoses

- Herpes simplex?
- Cellulitis?
- Bacterial resistance if previous antibiotic use?

Recurrent impetigo

- Send a skin swab.
- Consider sending a nasal swab.
- De-colonise: antiseptic body wash, nasal ointment or both, and personal hygiene measures to remove the bacteria causing the infection from the skin.